Father Muller Medical College Hospital

(A unit of Father Muller Charitable Institutions)
Father Muller Road, Kankanady, Mangaluru - 575 002, India
DEPT. OF RADIO-DIAGNOSIS & IMAGING

MR - 14E

Patient Name	: [redacted]	Age / Gender	: 51 Years / Female
Reg No	: 170117631 / IP 795370	Ward / Bed No	: O WARD / O Sur F 44
Request No	: 779233	Referred by	: Dr JOHN JOSEPH S. MARTIS MBBS,MS \ SURGERY UNIT B
Requested Date and time	: 31-10-2019/12:32	Reported date and time	: 01-11-2019 / 11:14
Cl. Diagnosis	:	Done By	: Dr LENON JASON DSOUZA

MAMMOGRAM BILATERAL

MAMMOGRAM - BILATERAL BREAST

Indication - Case of Carcinoma right breast.
- No previous mammograms available for comparison

- Craniocaudal and mediolateral views are studied

- Breast composition : Predominantly fatty breast pattern noted

Right breast

- Lobulated high density mass noted in inner lower quadrant of right breast.

- 2.8x2.1cms well defined smooth margins noted in high density mass lesion noted in upper quadrant in periaxillary region measuring 4.5x3.2cms.

- Few prominent right axillary lymphnodes noted with few showing loss of fatty hilum.

- No evidence of micro/macrocalcification

- No evidence of skin/trabecular thickening

- No evidence of skin/ nipple retraction.

Left breast

- Small high density opacity noted in outer upper quadrant of left breast- likely intramammary lymphnode.

- Prominent left axillary lymphnodes noted with maintained fatty hilum.

- No evidence of skin/trabecular thickening

- No evidence of skin/ nipple retraction.

IMPRESSION: Known case of Carcinoma right breast, present study shows-
- BI-RADS - VI.

Dr. LENON JASON DSOUZA
MBBS, MD, DNB, FVIR
ASSISTANT PROFESSOR

DAP28346 - 01-11-2019/11:14

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DEPT. OF RADIO-DIAGNOSIS & IMAGING

MR - 14E

Patient Name	: [redacted]	Age / Gender	: 59 Years / Female
Reg No	: 14/170086279 /	Ward / Bed No	: / --
Request No	: 705585	Referred by	: Dr CLEMENT R.S. D SOUZA MBBS,DNB \ MEDICINE UNIT D
Requested Date and time	: 09-05-2019/07:53	Reported date and time	: 09-05-2019 / 11:20
Cl. Diagnosis	:	Done By	: Dr LENON JASON DSOUZA

MAMMOGRAM SINGLE

MAMMOGRAM - LEFT BREAST

Indication - Follow up - Known case of carcinoma right breast.
Post operative status (10 years ago), on follow up.

- Craniocaudal and mediolateral views are studied

- Breast composition : Type A : predominantly fatty breast

- Lobulated mass is noted at 12 - 1 O'clock position of left breast with high density measuring approximately 30x23mm.

- The lesion was correlated on sonomammogram it appears hypoechoic with evidence of fine pleomorphic calcification within.

- The lesion shows posterior acoustic enhancement.

- No evidence of skin/trabecular thickening

- No evidence of skin/nipple retraction

- Multiple left axillary lymphnodes

- An intramammary lymphnode is seen adjacent to the mass

IMPRESSION:

KNOWN CASE OF CARCINOMA RIGHT BREAST, ON FOLLOW UP
PRESENT MAMMOGRAM SHOWS :

- LOBULATED MASS IN THE LEFT BREAST AS DESCRIBED - SUSPICIOUS FOR MALIGNANCY - BI-RADS - CATEGORY 4 C.
SUGGESTED FNAC CORRELATION.

Dr. LENON JASON DSOUZA
MBBS, MD, DNB, FVIR
ASSISTANT PROFESSOR

VIC10675 - 09-05-2019/11:20

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DEPT. OF RADIO-DIAGNOSIS & IMAGING

MR - 14E

Patient Name	: [redacted]	Age / Gender	: 59 Years / Female
Reg No	: 14/170086279 /	Ward / Bed No	: / --
Request No	: 705585	Referred by	: Dr CLEMENT R.S. D SOUZA MBBS,DNB \ MEDICINE UNIT D
Requested Date and time	: 09-05-2019/07:53	Reported date and time	: 09-05-2019 / 11:20
Cl. Diagnosis	:	Done By	: Dr LENON JASON DSOUZA

MAMMOGRAM SINGLE

MAMMOGRAM - LEFT BREAST

Indication - Follow up - Known case of carcinoma right breast.
Post operative status (10 years ago), on follow up.

- Craniocaudal and mediolateral views are studied

- Breast composition : Type A : predominantly fatty breast

- Lobulated mass is noted at 12 - 1 O'clock position of left breast with high density measuring approximately 30x23mm.

- The lesion was correlated on sonomammogram it appears hypoechoic with evidence of fine pleomorphic calcification within.

- The lesion shows posterior acoustic enhancement.

- No evidence of skin/trabecular thickening

- No evidence of skin/nipple retraction

- Multiple left axillary lymphnodes

- An intramammary lymphnode is seen adjacent to the mass

IMPRESSION:

KNOWN CASE OF CARCINOMA RIGHT BREAST, ON FOLLOW UP
PRESENT MAMMOGRAM SHOWS :

- LOBULATED MASS IN THE LEFT BREAST AS DESCRIBED - SUSPICIOUS FOR MALIGNANCY - BI-RADS - CATEGORY 4 C.
SUGGESTED FNAC CORRELATION.

Dr. LENON JASON DSOUZA
MBBS, MD, DNB, FVIR
ASSISTANT PROFESSOR

VIC10675 - 09-05-2019/11:20

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Patient Name	: [redacted],	Age / Gender	: 55 Years / Female
Reg No	: 14/200042396 /	Ward / Bed No	: / --
Request No	: 856620	Referred by	: Dr VINODA KUMAR B. MBBS,DNB \ SURGERY UNIT B
Requested Date and time	: 06-10-2020/12:35	Reported date and time	: 06-10-2020 / 17:20
Cl. Diagnosis	:	Done By	: Dr RITHI MELISSA D SILVA

MAMMOGRAM BILATERAL

MAMMOGRAM - BILATERAL BREAST

Indication - Lump in left breast
- No previous mammograms available for comparison

- Craniocaudal and mediolateral views are studied

- Breast composition : Scattered areas of fibroglandular density noted.

Right breast
- Well defined oval opacity noted in retroareolar location.

- No asymmetry/architectural distortion

- No evidence of micro/macrocalcification

- No evidence of skin/trabecular thickening

- No evidence of skin/nipple retraction

- No evidence of axillary lymphadenopathy

USG correlation reveals- Well defined, wider than taller, hypoechoic lesion at 9'o' clock position with no internal vascularity/ calcification- likely fibroadenoma.

Left breast
- Well defined opacity noted in upper quadrant of left breast posteriorly in MLO view.

- No asymmetry/architectural distortion

- No evidence of micro/macrocalcification

- No evidence of skin/trabecular thickening

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DEPT. OF RADIO-DIAGNOSIS & IMAGING

MR - 14E

Patient Name	: [redacted]	Age / Gender	: 57 Years / Female
Reg No	: 14/190078007 /	Ward / Bed No	: / --
Request No	: 805305	Referred by	: Dr SUSAN DSOUZA MBBS,MD
Requested Date and time	: 13-01-2020/08:04	Reported date and time	: 13-01-2020 / 12:42
Cl. Diagnosis	:	Done By	: Dr BASAVARAJ N BIRADAR

MAMMOGRAM BILATERAL

MAMMOGRAM - BILATERAL BREAST

Indication - Screening

- Craniocaudal and mediolateral views are studied

- Breast composition : Scattered areas of fibroglandular density noted.

Left breast :

- Oval radio opacity with smooth margins and microcalcifications noted in upper outer quadrant of left breast.

USG correlation shows- A Oval hypoechoic lesion wider than taller measuring 8x4mm noted in upper outer quadrant in periareolar region approximately 1cm from nipple at 1'o' clock position. No evidence of internal vascularity. Few small linear hyperechoic areas noted within the lesion.

- Few enlarged axillary lymphnodes noted with maintained fatty hilum largest measuring 13x7mm.

Right breast

- Focal oval radio opacity with macro calcification noted in upper outer quadrant of right breast.

USG correlation shows- A oval hypoechoic lesion with smooth margins measuring 6x4mm noted at 9'o'clock position with a macro calcification and posterior shadowing, approximately 2.5cms from nipple areolar region.

- Few enlarged axillary lymphnodes noted with maintained fatty hilum, largest measuring 9x5mm.

IMPRESSION:
- AN OVAL RADIO OPACITY WITH MACRO CALCIFICATION IN UPPER OUTER QUADRENT OF RIGHT BREAST. (BI RADS-II).
- AN OVAL RADIO OPACITY WITH FEW MICRO CALCIFICATIONS IN UPPER OUTER QUADRANT OF LEFT BREAST- BI RADS- IV. SUGGESTED FNAC CORRELATION.
- BILATERAL REACTIVE AXILLARY LYMPHNODES.

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